



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : اميره محمد شحات عبدالرحمن

المحافظة : القليوبيه الحي / المدينة : شبين القناطر

رقم البطاقة : 28309201404807 رقم التليفون : 01244649997 تاريخ تسجيل الإستمارة : 19-09-2026

العمر (تاريخ الميلاد) : 20-09-1983 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : جراحات أورام تدخل جراحى لاستئصال كامل /تحفظى للثدى +تفريغ قسم العلاج : الغدد الليمفاوية و/ او الغدد الحارسة [13155]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: اميره محمد شحات عبدالرحمن National ID: 28309201404807	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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Mastectomy or breast-conserving surgery with axillary lymph node dissection and/or sentinel lymph node biopsy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستاف
روستاف
11مراجعه
9-12مبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةALJUMHURIYAH CHARTER
FOR WOMEN'S
HEALTH

09/13/2026

MDT FORM

Name:	ليلى محمد شحات عبد الرحمن	MRN:	13693
Age:	43 Y	ID:	28309201404807
Menopausal Sta	MENSTURATION	Phone:	
NRS SCORE:		Marital Status:	MARRIED
BMI:		Family History:	
CO Morbidity:	FREE	Reason Of MDT:	NEW CASE

Clinical Data:

BX: FIBROEPITHELIAL LESION SUGGESTIVE FIBRO ADENOMA

ER: PR: HER2: KI67:

Sonomammography:

LEFT UTQ LESION AT 11-12 OCLOCK SHOWING CYSTIC CHSNGES AND
VASCULARITY
CLINICAL PALPABLE

Mets Workup:

History:

Recommendations:

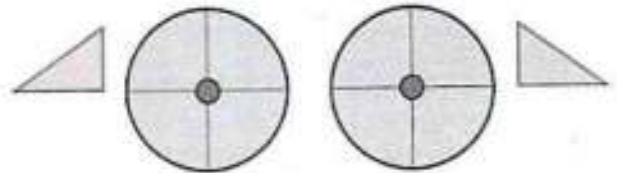
FOR LUMPECTOMY

MDT Outcome & Plan

FOR LUMPECTOMY

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TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هادي عزت	د. محمد نجيب				

Radiological exam

Breast Density	
Mammography	
Ultrasound	 جمهورية مصر العربية بطاقة تحليل الشخصية اميره محمد شحات عبدالرحمن عرب الشعاره مرکز شبين القناطر - القليوبية  ٢٨٣.٠٩٢٠١٤.٤٨.٧
	KL3753456
	Solid Cyst Complex cyst Inflammatory changes Dilated duct Skin changes Nipple involvement Chest wall
SITE	UOQ UIQ LOQ LIQ Retro-areolar Axillary tail N/A
Size (largest)	No mass (T0) 0-2 cm (T1) 2-5 cm (T2) >5cm (T3) Could not be measured (Tx) N/A
Number	Single Multifocal Multicentric N/A
Axillary nodes	Non specific Indeterminate Pathological N/A
BIRADS	1 - 2 - 3 - 4 - 5 - 6
Biopsy	
Remarks	

Fast Density	
Mammography	A
	Normal
	Mar-



٢٠٢٤/٠٢ ٢٨٣٠٩٢٠١٤٠٤٨٠٧
ر.ب. منزل
انثى
امينة
متزوجة
ايمن عبدالعليم حسين عبدالعليم
البطاقة سارية حتى ٢٠٣١/٠٢/١٨





	Inflammatory changes
	Dilated duct
	Skin changes
	Nipple involvement
	Chest wall
SITE	UOQ
	UIQ
	LOQ
	LIQ
	Retro-areolar
	Axillary tail
	N/A
Size (largest)	No mass (T0)
	0-2 cm (T1)
	2-5 cm (T2)
	>5cm (T3)
	Could not be measured (Tx)
	N/A
Number	Single
	Multifocal
	Multicentric
	N/A
Axillary nodes	Non specific
	Indeterminate
	Pathological
	N/A
BIRADS	
Biopsy	
Remarks	1-2-3-4-5-6

MHMED, AMERA Patient ID: 10110324009755 DOB: 20-Sep-1983 Date: 08-Oct-2025

Patient Lastname: Mhmed

Patient Firstname: Amera

HIS Patient ID: 10110324009755

RIS Patient ID: 359892

Date of Birth: 20/09/1983

Procedure: 76499, MAMMOGRAPHY WITH U/S

Medical Question:

Clinical Comment:

===== REPORT TEXT =====

BILATERAL DIGITAL MAMMOGRAPHIC EXAMINATION AND COMPLEMENTARY ULTRASOUND

" ACR: B

" No spiculated mass lesions or suspicious microcalcifications.

" There is an increase in the size of the left breast UIQ partially obscured medium density lesion demonstrating few overlying calcific foci; complementary US confirmed the increase in size of the previously described 11-12:00 circumscribed lobulated isoechoic lesion showing few cystic changes and mild vascularity currently measuring 35x22 in comparison to the previous measurement of 24x14mm

" Right breast anechoic cysts are noted.

" Normal skin thickness and nipple/areola complexes.

" No pathologically axillary lymphadenopathy.

OPINION:

" Progression in the size of the left lesion, BIRADS 4a for biopsy.

" Right breast study simple cyst,s BIRADS 2.

===== REPORT TEXT ENDE =====

1011.244-9700

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology



جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Exam Information:

Patient Name: Amera Mhmed	Date of birth: 20/09/1983
Arabic Name: اميرة محمد شحات	Patient Gender: F
Patient RIS ID: 359892	Exam Date: 09/10/2024
Patient HIS ID: 10110324009775	
Written by: Dr. Fifi Samy	Approved by: Dr. Fifi Samy

**BILATERAL DIGITAL MAMMOGRAPHIC EXAMINATION AND
COPPLEMENTARY ULTRASOUND**

- ACR: B
- No spiculated mass lesions or suspicious microcalcifications.
- Left breast UIQ partially obscured medium density lesion corresponding by US to 11-12 o'clock zone B circumscribed parallel oriented hypoechoic lesion measuring about 1.4 x 2.4 cm with related prominent clear duct.
- No right breast solid or cystic lesions.
- Normal skin thickness and nipple/areola complexes.
- No axillary lymphadenopathy.

OPINION:

- Left breast UIQ benign looking lesion likely fibroadenoma BIRADS 3.
- Normal right breast study BIRADS 1.

Patient Name: أميرة محمد شحات عبدالرحمن
MRN: 10110324009755
D.O.B: 20/09/1983
IC No.: 28309201404807

Location: External Clinic
Sex: Female

Surgical Pathology-Diagnosis

Biopsy

Specimen Id 2510280253
Accession No P-25-13578
Sample Type TISSUE

Collection Date 28/10/2025 08:34 AM
Received Date 28/10/2025 09:03 AM
Doctor

CLINICAL HISTORY

Left breast upper inner quadrant mass (BIRADs 4a).

GROSS DESCRIPTION

Fragmented tissue cores from 0.5 to 1 cm long, totally submitted.

MICROSCOPIC DESCRIPTION

Breast tissue cores showing fibroepithelial lesion formed of proliferating epithelial and stromal elements. Fibrous stroma proliferates around patent ductal spaces. Some ducts show proliferation of epithelial cells showing nuclear overlapping, indistinct cell borders and irregularly sized and shaped fenestrations and slit-like spaces with secondary lumina. These proliferating hyperplastic cells are lacking the cytologic atypia (usual ductal hyperplasia = UDH). No atypia or malignancy. Picture is suggestive of fibroadenoma, recommended for confirmatory excision.

DIAGNOSIS

Left breast upper inner quadrant mass, Tru cut biopsy: Fibroepithelial lesion, Suggestive of fibroadenoma, recommended for confirmatory excision.

Category

Benign Tumor

Assistant Pathologist:
Dr. Nada Saber

Registrar:

Signed by:
Dr. Noura A. A. Ebrahim, MD (Lecturer)

Reported and electronically endorsed by Dr. Nada Saber



Name: Amira Mohamed Shahat

Date: 27 August 2024

MAMMOGRAPHY OF LEFT BREAST

Findings:

- Overall breast composition: there are scattered areas of fibroglandular densities (ACR category b).
- A well-defined retro-areolar radio-opaque shadow is noted likely fibroadenoma for US correlation.
- Left breast displays normal skin thickness with normal appearance of the nipple and subareolar complex.
- Normal appearance of the subcutaneous and retromammary fat.
- No suspicious spiculated masses, architectural distortion or asymmetric densities.
- No suspicious clusters of microcalcifications are noted.
- No axillary lymphadenopathy.

Impression:

- Left breast well-defined radio-opaque shadow for US correlation.

MUCH OBLIGED

Dr. Mohamed Khedr, MSc

Ass.Lecturer of Radiology Ainshams University

BI-RADS classification according to the American College of Radiology

Category	Management	Likelihood of cancer
0 Incomplete	Need further evaluation	Not applicable
1 Negative	Routine screening	0%
2 Benign	Routine screening	0%
3 Probably benign	Short interval follow up	< 2%
4 Suspicious abnormality	Tissue diagnosis	2 - 95%
5 Highly suggestive of malignancy	Tissue diagnosis	> 95%
6 Biopsy proven malignancy	Surgical excision when clinically appropriate	



MHMED, AMERA Patient ID: 10110324009755 DOB: 20-Sep-1983 Date: 08-Oct-2025

Patient Lastname: Mhmed

Patient Firstname: Amra

HIS Patient ID: 10110324009755

RIS Patient ID: 359892

Date of Birth: 20/09/1983

Procedure: 76499, MAMMOGRAPHY WITH U/S

Medical Question:

Clinical Comment:

REPORT TEXT

BILATERAL DIGITAL MAMMOGRAPHIC EXAMINATION AND COMPLEMENTARY ULTRASOUND

" ACR: B

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" Right breast anechoic cysts are noted.

" Normal skin thickness and nipple/areola complexes.

" No pathologically axillary lymphadenopathy.

OPINION:

" Progression in the size of the left lesion, BIRADS 4a for biopsy.

" Right breast study simple cysts, BIRADS 2.

REPORT TEXT ENDE

- OPINION:
- Left breast UIQ benign looking lesion likely fibroadenoma BIRADS 3.
 - Normal right breast study BIRADS 1.

- ACR: B
- No spiculated mass lesions or suspicious microcalcifications.
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**BILATERAL DIGITAL MAMMOGRAPHIC EXAMINATION AND
COMPLEMENTARY ULTRASOUND**

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Patient RIS ID: 359892	Patient HIS ID: 10110324009775
Written by: Dr. Fifi Samy	
Approved by: Dr. Fifi Samy	Exam Date: 09/10/2024
Date of birth: 20/09/1983	Patient Gender: F

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للسرطان
قسم الأشعة التشخيصية والتداخلية

1.11.2024-2400

Reported and electronically endorsed by Dr. Nada Saber

Dr. Nada Saber

Assistant Pathologist:

Registrar:

Signed by:

Dr. Noura A. A. Ebrahim, MD (Lecturer)

Benign Tumor

Category

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Accession No	P-25-13578	Received Date	28/10/2025 09:03 AM
Sample Type	TISSUE	Doctor	

Surgical Pathology-Diagnosis

Biopsy

IC No.:	28309201404807
D.O.B:	20/09/1983
MRN:	10110324009755
Patient Name:	اميرة محمد شحات عبدالرحمن
Location:	External Clinic
Sex:	Female

6	Biopsy proven malignancy	Surgical excision when clinically appropriate	
5	Highly suggestive of malignancy	Tissue diagnosis	> 95%
4	Suspicious abnormality	Tissue diagnosis	> 2-95%
3	Probably benign	Short interval follow up	< 2%
2	Benign	Routine screening	0%
1	Negative	Routine screening	0%
0	Incomplete	Need further evaluation	Not applicable
	Category	Management	Likelihood of cancer

BI-RADS classification according to the American College of Radiology

Ass. Lecturer of Radiology Ains Shams University

Dr. Mohamed Khedr, MSc

MUCH OBLIGED

> Left breast well-defined radio-opaque shadow for US correlation.

Impression:

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- Normal appearance of the subcutaneous and retromammary fat.
- No suspicious spiculated masses, architectural distortion or asymmetric densities.
- No suspicious clusters of microcalcifications are noted.
- No axillary lymphadenopathy.

Findings:

MAMMOGRAPHY OF LEFT BREAST

Date: 27 August 2024

Name: Amira Mohamed Shahat



El Mahalawy
Radiology
Center

المركز الرئيسي

مركز

MHMED, AMERA Patient ID: 10110324009755 DOB: 20-Sep-1983 Date: 08-Oct-2025
Patient Lastname: Mhmed
Patient Firstname: Amera
HIS Patient ID: 10110324009755
RIS Patient ID: 359892
Date of Birth: 20/09/1983

Procedure: 76499, MAMMOGRAPHY WITH U/S
Medical Question:
Clinical Comment:

===== REPORT TEXT =====
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OPINION:
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" Right breast study simple cysts, BIRADS 2.

===== REPORT TEXT ENDE =====

09/13/2026

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منظمة الصحة العالمية
World Health Organization

مستشفى جامعة القاهرة
Cairo University Hospital

9-14



MDT FORM

Name:	43 Y	MRN:	13693
Age:	43 Y	ID:	28309201404807
Menopausal Sta	MENSTRUATION	Phone:	
NRS SCORE:		Marital Status:	MARRIED
BMI:		Family History:	
CO Morbidity:	FREE	Reason of MDT:	NEW CASE

Clinical Data:

BX: FIBROEPITHELIAL LESION SUGGESTIVE FIBRO ADENOMA
ER: PR: HER2: KI67:

Sonomammography: LEFT UIQ LESION AT 11-12 OCLOCK SHOWING CYSTIC CHNGES AND

VASCULARITY

CLINICAL PALPABLE

Mets Workup:

History:

Recommendations:

FOR LUMPECTOMY

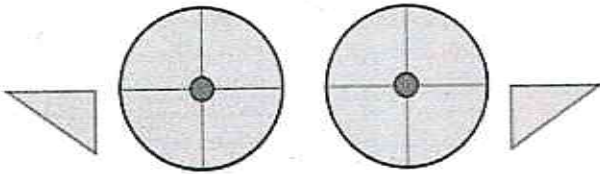
MDT Outcome & Plan

FOR LUMPECTOMY

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
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